

1. Administrative & Study Identification

Full Study Title: A Clinical Study of Molnupiravir to Prevent Severe Illness From Coronavirus Disease 2019 (COVID-19) in People Who Are High Risk (MK-4482-023) **Official Title:** A Phase 3, Randomized, Placebo-Controlled, Double-Blind Clinical Study to Evaluate the Efficacy and Safety of Molnupiravir (MK-4482) in Non-Hospitalized Adults With COVID-19 at High Risk for Disease Progression **Short Title/Acronym:** MOVE-NOW Study **Protocol Number:** MK-4482-023 **NCT Number:** NCT06667700 **Posting ID:** 908009312679210814 **Sponsor Name:** Merck (Merck Sharp & Dohme LLC in collaboration with Ridgeback Biotherapeutics) **Investigational Product:** Molnupiravir (LAGEVRIO™) / MK-4482 **ATC Code:** J05AB18 **Phase of Development:** Phase 3 **Trial Status:** Recruiting (Currently recruiting participants) **Medical Monitor:** Merck Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the efficacy and safety of orally administered molnupiravir compared with placebo in preventing severe illness (disease progression) from COVID-19 in non-hospitalized adults at high risk. **Secondary Objectives:** To assess SARS-CoV-2 viral load, the time to sustained alleviation of symptoms without relapse, the percentage of participants with undetectable SARS-CoV-2 RNA, and the development of post-acute sequelae of COVID-19 (PASC, or long COVID) over an extended follow-up period.

2.2 Background & Rationale COVID-19 remains a leading cause of hospitalization and death globally. People with certain health conditions or factors (like advanced age, immunocompromised states, or multiple comorbidities) have a high risk of severe illness. Some of these high-risk individuals are unable to take other antiviral treatments like nirmatrelvir/ritonavir (NMV/r) due to drug-drug interactions, allergies, or inaccessibility. This study evaluates molnupiravir—a medicine designed to stop the COVID-19 virus from multiplying—to provide a critical treatment option for this vulnerable population to prevent severe outcomes.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Placebo-controlled **Blinding:** Double-blind **Randomization:** Randomized

3.2 Planned Enrollment & Duration Targeted Enrollment: 3,082 participants **Number of Sites:** Multicenter global study across 25 markets **Estimated Study Start:** 02-Dec-2024 **Estimated Primary Completion:** 26-Jan-2031

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Age ≥ 18 years of any sex/gender. 2. Documentation of SARS-CoV-2 infection with sample collection ≤ 4 days prior to randomization. 3. Has initial onset of signs/symptoms attributable to COVID-19 for ≤ 4 days prior to the day of randomization and ≥ 2 of the following signs/symptoms on the day of randomization: cough, sore throat, nasal congestion, shortness of breath or difficulty breathing with exertion, muscle or body aches, fatigue, fever $>38.0^{\circ}\text{C}$ or chills, nausea or vomiting or diarrhea, change in sense of smell or change in sense of taste, or headache. 4. High risk for disease progression, defined by ≥ 1 of the following characteristics: advanced age (≥ 75 years), being immunocompromised, neurocognitive or physical disability, or having ≥ 3 risk characteristics (e.g., chronic lung disease, obesity with BMI ≥ 35 , diabetes). 5. Is unable or unwilling to receive treatment with nirmatrelvir/ritonavir (NMV/r) due to drug-drug interactions, severe hepatic impairment, prior adverse reactions/hepatotoxicity, uncontrolled HIV, drug shortage/inaccessibility, known resistance, or unwillingness. *Inclusion Note:* Participants may receive remdesivir as standard of care in addition to molnupiravir or placebo.

4.2 Exclusion Criteria 1. Currently hospitalized or is expected to need hospitalization for COVID-19 imminently. 2. Has received or plans to receive SARS-CoV-2 directed oral antivirals or monoclonal antibodies for current episode of COVID-19 (other than study intervention and remdesivir as standard of care). 3. Has ≥ 1 signs or symptoms attributable to severe or critical COVID-19: shortness of breath at rest, respiratory rate ≥ 30 breaths per minute, heart rate ≥ 125 beats per minute, peripheral oxygen saturation (SpO₂) $\leq 93\%$ on room air (or on supplemental oxygen for a non-COVID reason which has not increased since COVID onset). 4. New or increasing need for supplemental oxygen: receiving >4 liters/minute supplemental oxygen due to COVID-19 OR on supplemental oxygen for a non-COVID reason which has increased due to COVID-19. 5. Has received a COVID-19 vaccine within 30 days prior to randomization. 6. Has a history of confirmed influenza, respiratory syncytial virus (RSV), or SARS-CoV-2 infection (excluding current infection) within 30 days prior to randomization. 7. Has known or suspected hypersensitivity to active or inactive ingredients of molnupiravir.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Molnupiravir (MK-4482) 800 mg (administered as two 400-mg tablets) or matching placebo every 12 hours. **Route of Administration:** Oral **Duration of Treatment:** 5 days

5.2 Concomitant & Prohibited Therapy Permitted: Remdesivir as standard of care; general supportive care for COVID-19 symptoms. **Prohibited:** Concurrent use of other investigational or authorized antiviral COVID-19 therapies or monoclonal antibodies.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening/Baseline	Day 0	Informed Consent, Medical History, SARS-CoV-2 Testing, Randomization, First Dose
Treatment Period	Days 1 - 5	Investigational Product Administration (BID), Symptom Diary, Safety Assessments
Interim	Day 29	Acute care visit evaluation, Efficacy Assessment, Initial Evaluation of PASC (Long COVID) symptoms
End of Study	Month ~5.5	Final Follow-Up, Extended Evaluation of PASC (Long COVID), Exit Interview

7. Outcome Measures (Endpoints)

7.1 Primary Endpoints

1. **Percentage of Participants Who Experienced One or More of Following Through Day 29:** All-cause Hospitalization (≥ 24 hours of acute care), All-cause Mortality, or COVID-19-related Medically-attended Visit (MAV).
2. **Percentage of Participants Who Experienced an Adverse Event (AE).**
3. **Percentage of Participants Who Discontinued Study Intervention Due to AE.**

7.2 Secondary & Exploratory Endpoints

1. **Time to Sustained Alleviation (without relapse)** of all selected 8 (5 prespecified and 3 determined by baseline prevalence) self-reported COVID-19 signs/symptoms.
2. **Change from Baseline in Severe Acute Respiratory Syndrome Coronavirus 2 (SARS-CoV-2) RNA Titer.**
3. **Percentage of Participants with Undetectable SARS-CoV-2 RNA.**
4. **Incidence of post-acute sequelae of COVID-19 (PASC)** during the extended follow-up.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Standard collection via patient self-reported diaries, remote check-ins, and scheduled clinical assessments to determine the percentage of participants experiencing AEs or discontinuing due to AEs. **Serious Adverse Events (SAEs):** Continuously monitored and reported through the

trial's safety database. **Data Monitoring Committee (DMC):** Independent external data monitoring committee oversight for safety and efficacy signals.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Modified Intent-to-Treat (mITT) population for primary efficacy outcomes; Safety Set for all AE/SAE evaluations. **Sample Size Justification:** Target enrollment of \$N=3,082\$ participants to adequately power the study for demonstrating a relative risk reduction in severe illness/hospitalization/mortality against placebo. **Handling of Missing Data:** Standard imputation methods for longitudinal symptom tracking and viral load assessments.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Routine onsite and remote monitoring of participating global sites to ensure protocol adherence and data integrity. **Audit Trail:** Secure EDC (Electronic Data Capture) systems with full audit trails for eCRF data entry and clarification forms.

11. Study Identifiers & Governance

Primary ID: MK-4482-023 **Registry Number:** NCT06667700 **Posting ID:** 908009312679210814 **Sponsor/Lead Organization:** Merck **Trial Status:** Recruiting **Study Title:** MOVE-NOW Study

12. Clinical Framework (COVID-19 Specific)

Disease Areas: Coronavirus Disease (COVID-19) **Preferred Name:** COVID-19 Virus Disease **Preferred UMLS Name:** Prevention of COVID-19 **MeSH Code:** D000086382 **SNOMED ID:** 840539006 **Medical Methodology:** Oral Antiviral Therapy (Viral Replication Inhibitor) **Optimization Target:** Prevention of severe disease progression / acute care visits

13. Study Procedures & Methodology

Management Pathway: Rapid initiation of oral antiviral therapy upon symptom onset **Education Protocol (HCP):** Site-level training on identifying high-risk patients ineligible for NMV/r **Education Protocol (Patient):** Guidance on symptom diary logging and

medication adherence **Quality Audit Mechanism:** Remote tracking of dosing adherence and symptom resolution timelines

14. Observation & Follow-Up Schedule

Total Duration: ~5.5 Months follow-up post-treatment **Onsite Visit Cadence:** Baseline, Day 29 (as per site protocol) **Remote Monitoring Frequency:** Weekly check-ins during acute phase, transitioning to monthly for long COVID evaluation **Checklist Review Interval:** Daily during the 5-day treatment period

15. Sign-off & Approval

Role	Name	Signature	Date	:---	:---	:---	:---	
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]					
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]					Lead
Statistician	[Name]	_____	[DD-MMM-YYYY]					